

ST. MARGARET'S REGIONAL MEDICAL CENTER

Department of Internal Medicine & Hospitalist Services
4850 Mercy Boulevard · Suite 200 · Springfield, IL 62704
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HOSPITAL DISCHARGE SUMMARY

Patient Name: Echevarría, Ester
Date of Birth: June 20, 1927 (Age 99 years)
Sex: Female
Medical Record #: MRN 8842-9635
Admission Date: September 14, 2019
Discharge Date: September 16, 2019
Length of Stay: 2 days
Attending Physician: Dr. Marcus L. Hendricks, MD, Internal Medicine

ADMITTING DIAGNOSIS

Mechanical fall with observation status for injury assessment; urinary tract infection.

DISCHARGE DIAGNOSES

1. Mechanical fall at home without fracture
2. Urinary tract infection (UTI), *E. coli* sensitive to ceftriaxone
3. Acute delirium, resolved
4. Alzheimer's disease
5. Atrial fibrillation, rate-controlled, on anticoagulation
6. Diabetes mellitus type II with diabetic nephropathy and retinopathy
7. Chronic kidney disease stage 1
8. Anemia of chronic kidney disease
9. Recurrent colorectal polyps, metastatic colorectal carcinoma on palliative chemotherapy (FOLFOX)
10. Hyperlipidemia
11. Hypertriglyceridemia
12. Metabolic syndrome
13. Osteoarthritis of bilateral knees
14. Chronic sinusitis
15. Viral sinusitis, resolving
16. Acute bronchitis, resolving

HOSPITAL COURSE

Ms. Echevarría is a 99-year-old Spanish-speaking female with extensive past medical history as above who presented to the Emergency Department on September 14, 2019, following a witnessed mechanical fall at her assisted living facility. The patient tripped over a rug and fell forward onto her hands and knees. There was no loss of consciousness, no head strike, and no seizure activity witnessed. The patient complained of bilateral knee pain and mild confusion.

In the Emergency Department, she was found to be alert but mildly confused compared to her baseline. Vital signs were stable with blood pressure 142/78 mmHg, heart rate 88 bpm (irregularly irregular), respiratory rate 18, oxygen saturation 96% on room air, and temperature 100.8°F. Physical examination revealed tenderness over bilateral knees without obvious deformity, ecchymosis, or effusion. No focal neurologic deficits were identified. Cardiovascular examination was consistent with her known atrial fibrillation.

Radiographic imaging of bilateral knees and pelvis showed no acute fracture or dislocation, with findings consistent with moderate degenerative joint disease. CT head without contrast demonstrated age-appropriate cerebral atrophy and chronic small vessel ischemic changes, but no acute intracranial hemorrhage or mass effect. Urinalysis was notable for pyuria, bacteriuria, and positive nitrites. Urine culture subsequently grew >100,000 CFU/mL *Escherichia coli* sensitive to ceftriaxone, ciprofloxacin, and trimethoprim-sulfamethoxazole.

Laboratory studies revealed: White blood cell count 11,200/ μ L with left shift, hemoglobin 10.2 g/dL (baseline 10-11), creatinine 1.1 mg/dL (baseline), INR 2.3 (therapeutic for atrial fibrillation), glucose 178 mg/dL. Liver function tests were within normal limits.

The patient was admitted to the internal medicine service for observation and treatment of UTI. Intravenous ceftriaxone 1 gram daily was initiated. Her acute confusion improved significantly within 24 hours of antibiotic therapy and hydration. Pain management was achieved with acetaminophen 650 mg every 6 hours as needed; NSAIDs were avoided given her renal disease and warfarin therapy. Physical therapy evaluated the patient and recommended continued assistance with ambulation and return to baseline functional status with walker use.

The patient's home medications were continued throughout hospitalization with careful monitoring of INR given antibiotic interaction with warfarin. Her atrial fibrillation remained rate-controlled on her home regimen of verapamil and digoxin. Blood glucose control was maintained with home insulin regimen.

By hospital day 2 (September 16), the patient had returned to her baseline mental status per family report, was afebrile for >24 hours, ambulatory with walker and assistance, and tolerating oral intake well. She was deemed appropriate for discharge back to her assisted living facility with close outpatient follow-up.

GOALS OF CARE DISCUSSION

An extensive goals-of-care discussion was held on September 15, 2019, with the patient's family, including her daughter Maria Echevarría (healthcare power of attorney) and son Roberto Echevarría. The discussion was conducted with the assistance of professional Spanish interpretation services to ensure the patient's participation to the extent possible given her moderate Alzheimer's disease.

The family confirmed that the patient has established DNR/DNI (Do Not Resuscitate/Do Not Intubate) status, documented in her advance directive dated March 2019. The family expressed understanding that at age 99 with multiple comorbidities including metastatic colorectal cancer, the focus of care is on comfort, quality of life, and

symptomatic management. The patient is currently receiving palliative chemotherapy (FOLFOX regimen) through her oncologist, Dr. Patricia Silverman, with the goal of disease control and maintaining functional status rather than cure.

The family wishes to continue the current treatment plan including chemotherapy as long as the patient tolerates it and maintains acceptable quality of life. They understand that should the patient experience clinical deterioration, significant decline, or intolerable side effects, the focus would shift entirely to comfort measures. Palliative care consultation has been offered and the family is considering this for future coordination.

All parties agreed that hospitalization should be avoided when possible, with management of acute issues at the assisted living facility when feasible. The patient should not undergo cardiopulmonary resuscitation, mechanical ventilation, or intensive care unit admission. Natural death is anticipated and accepted by patient and family. This discussion was documented in the electronic medical record and communicated to the patient's outpatient providers and assisted living facility nursing staff.

DISCHARGE MEDICATIONS

The following medications were reconciled and are to be continued as prescribed:

Medication	Dose/ Strength	Route/Frequency	Indication
Warfarin Sodium	5 mg	Oral, once daily at 5:00 PM	Atrial fibrillation anticoagulation (target INR 2.0-3.0)
Digoxin	0.125 mg	Oral, once daily in morning	Atrial fibrillation rate control
Verapamil Hydrochloride	40 mg	Oral, three times daily	Atrial fibrillation rate control, blood pressure management
Galantamine	4 mg	Oral, twice daily	Alzheimer's disease, cognitive support
Insulin Human Isophane 70 UNT/ML / Regular Insulin Human 30 UNT/ML (Humulin 70/30)	Per sliding scale	Subcutaneous injection, twice daily before breakfast and dinner	Diabetes mellitus type II
Simvastatin	10 mg	Oral, once daily at bedtime	Hyperlipidemia, cardiovascular risk reduction
Epoetin Alfa (Epogen)	4,000 units/mL, 1 mL injection	Subcutaneous, three times weekly (Monday, Wednesday, Friday)	Anemia associated with chronic kidney disease
Acetaminophen	325 mg, 1-2 tablets	Oral, every 6 hours as needed	Pain (osteoarthritis, general discomfort); maximum 3,000 mg per day
Naproxen Sodium	220 mg	Oral, twice daily as needed with food	Osteoarthritis pain (use sparingly given age, renal disease, and warfarin; acetaminophen preferred)
Ceftriaxone	1 gram	Intravenous, once daily	Urinary tract infection; NEW MEDICATION to complete 7-day course

		(through September 20, 2019). To be administered by facility nursing.
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Chemotherapy Medications (Administered by Oncology): The patient continues on FOLFOX regimen for metastatic colorectal carcinoma under the care of Dr. Patricia Silverman, Oncology. Current cycle medications include leucovorin 100 mg injection and oxaliplatin 5 mg/mL (10 mL) injection per oncology protocol. Next scheduled chemotherapy session is September 23, 2019.

Medication Changes During Hospitalization: Ceftriaxone was initiated for UTI treatment. Amoxicillin/clavulanate 250 mg/125 mg, which the patient was taking for sinusitis, was discontinued as the acute bronchitis and viral sinusitis have resolved. All other home medications were continued without change.

Special Instructions: INR should be checked on September 18, 2019 (48 hours post-discharge) due to drug-drug interaction between warfarin and ceftriaxone. Results should be reported to Dr. Angela Ramirez, primary care physician. Warfarin dose adjustment may be needed.

DISCHARGE CONDITION

The patient is discharged in stable condition. She is alert and oriented to person, though with baseline cognitive impairment from Alzheimer's disease. Vital signs are stable, afebrile. Ambulatory with walker and standby assistance, consistent with baseline functional status. Tolerating regular diet. Pain is well-controlled with acetaminophen.

DISCHARGE DISPOSITION

The patient is discharged to her home at Sunrise Assisted Living Facility, 2240 Meadowbrook Lane, Springfield, IL 62704, where she resides with 24-hour nursing support and assistance with activities of daily living.

DISCHARGE INSTRUCTIONS

Activity:

- Ambulate with walker and caregiver assistance at all times
- Remove tripping hazards (rugs, clutter) from living environment
- Continue physical therapy as arranged by assisted living facility
- No driving

Diet:

- Regular diet as tolerated
- Diabetic-appropriate meals with carbohydrate consistency
- Adequate hydration, encourage 1,500 mL fluid intake daily

Wound Care:

- Minor abrasions on bilateral knees may be cleansed with soap and water daily; apply thin layer of petroleum jelly and cover with gauze if needed for comfort

Monitoring:

- Monitor for signs of infection: fever, increased confusion, urinary symptoms
- Monitor for bleeding or bruising given anticoagulation therapy

- Daily blood glucose monitoring per facility protocol
- INR check on September 18, 2019
- Weekly weights to monitor fluid status

Warning Signs - Seek Medical Attention if:

- Fever >100.4°F
- Recurrent falls
- Increased confusion or lethargy beyond baseline
- Chest pain, shortness of breath, or palpitations
- Signs of bleeding: blood in urine or stool, excessive bruising, bleeding that won't stop
- Inability to take medications or retain oral intake
- Any other concerning symptoms

FOLLOW-UP APPOINTMENTS

Primary Care Physician:	Dr. Angela Ramirez, Springfield Family Medicine, 1640 West Monroe Street, Springfield, IL 62704, Phone: (217) 555-3400 Appointment: September 23, 2019 at 10:00 AM
Oncology:	Dr. Patricia Silverman, Regional Cancer Center, 4900 Mercy Boulevard, Suite 150, Springfield, IL 62704, Phone: (217) 555-7800 Chemotherapy session: September 23, 2019 at 1:00 PM (previously scheduled)
Cardiology:	Dr. Thomas Chen, Heartland Cardiology Associates, 530 North Grand Avenue East, Springfield, IL 62702, Phone: (217) 555-6200 Follow-up in 6 weeks for atrial fibrillation management (patient may call to schedule)
Laboratory:	INR check on September 18, 2019 at assisted living facility; results to be sent to Dr. Ramirez for warfarin dose adjustment if needed

PENDING STUDIES

None at time of discharge. All necessary laboratory and radiographic studies have been completed.

PATIENT/FAMILY EDUCATION

Discharge instructions were reviewed in detail with the patient's daughter Maria Echevarría (healthcare POA) via professional Spanish interpreter on September 16, 2019. Written discharge instructions in Spanish were provided. The family verbalized understanding of:

- The need to complete the full 7-day course of intravenous ceftriaxone for UTI
- Fall prevention strategies and continued use of walker with assistance
- Warning signs requiring medical attention
- Importance of INR monitoring and follow-up appointments
- Goals of care and established DNR/DNI status

- Palliative nature of ongoing chemotherapy

The family was encouraged to contact the primary care physician or return to the Emergency Department for any concerns. All questions were answered to satisfaction.

Dictated and Electronically Signed by:

Marcus L. Hendricks, MD, FACP

Internal Medicine, Hospitalist Service

St. Margaret's Regional Medical Center

NPI: 1234567890

Date/Time: September 16, 2019 at 2:45 PM

Electronically signed and authenticated. No handwritten signature required per hospital policy for electronic medical records.

Copies sent to: Dr. Angela Ramirez (PCP), Dr. Patricia Silverman (Oncology), Dr. Thomas Chen (Cardiology), Sunrise Assisted Living Facility (Nursing Staff)